Uphalyte
Oral Rehydration Salts

DESCRIPTION

UPHALYTE Oral Rehydration Salts (Natural):
White colour granules, when dissolve in water, forms a natural flavor, clear solution.
UPHALYTE Oral Rehydration Salts (Orange):
White to off-white colour granules, when dissolve in water, forms an orange flavor and orange colour solution.

Each sachet contains sodium chloride 525mg, sodium bicarbonate 425mg, potassium chloride 375mg, and glucose anhydrous 6.25gm. When reconstituted (1 sachet in 250ml of water), the ionic concentration is as follows:
Na^+ 56mmol/l, K^+ 20mmol/l, Cl^- 56mmol/l, HCO_3^- 20mmol/l, Glucose 139mmol/l.

INDICATIONS

For replacement of water and electrolyte loss associated with diarrhoea and vomiting.

PHARMACODYNAMICS

Oral rehydration salts are given orally to prevent or treat dehydration due to acute diarrhoea. Essential water and salts are lost in stools and vomitus, and dehydration results when blood volume is decreased because of fluid loss from the extracellular fluid compartment. Preservation of the facilitated glucose-sodium cotransport system in the small-bowel mucosa is the rationale of the oral rehydration therapy. Glucose is actively absorbed in the normal intestine and carries sodium with it in about an equimolar ration. Therefore, there is a greater net absorption of an isotonic salt solution with glucose than one without it. Potassium replacement during acute diarrhoea prevents below-normal serum concentrations of potassium, especially in children, in whom stool potassium losses are higher than in adults. Bicarbonates are effective in correcting the metabolic acidosis caused by diarrhoea and dehydration.

PHARMACOKINETICS

Uphalyte Oral Rehydration Salts contains anhydrous glucose, sodium chloride, potassium chloride and sodium bicarbonate which are essential for the body. Sodium chloride is involved in maintaining the osmotic tension of the blood and tissues. Sodium bicarbonate is involved in the management of acid-base disturbances due to diarrhoea. Potassium is involved in muscular contraction, conduction of nerves impulses, enzyme action and cell membrane functions. Glucose provides a rapidly available source of energy. The basis for oral rehydration is the glucose-facilitated sodium absorption in the small intestine. In severe diarrhoea, passive sodium diffusion and the active sodium pump mechanism are not functioning properly. However, the glucose-facilitated sodium absorption remains intact as long as glucose concentrations remain between 80-120mmol/L. Peak effect occurs between 8-12 hours after administration.

DOSAGE

Add 250ml of boiled, cooled water to the contents of one sachet of Uphalyte Oral Rehydration Salts. Stir or shake well for 2 to 3 minutes to dissolve.

Use as advised by your medical practitioner. As a general guideline, the following regime may be used:

	Serving in the 1st 2 hours	Serving in the 24 hours
Adults	4	12
Children 2 to 12 years	3	6
Infant up to 2 years of age	1	1½
	(Each serving equals 250ml reconstituted Uphalyte Oral Rehydration Salts)	

Infants and children should be given after every loose motion.

PATIENT COUNSELLING NOTES

- Do not boil the solution or add extra salt or sugar.
- Only mix with freshly boiled and cooled water. Do not mix with fizzy drinks, juice or milk.
- The solution should be prepared fresh daily.
- Do not use if granules are wet.
- If nausea or vomiting are present, and in infants and young children, drink the solution slowly in sips at shorter intervals to reduce vomiting and improve absorption.
- Eat soft foods such as cereals, bananas, cooked peas, beans and potatoes to maintain nutrition.
- Drink water between doses of rehydration solution.
- Check with your medical practitioner if diarrhoea does not improve in 1-2 days or becomes worse during treatment with ORS, or if signs of severe dehydration occur.

OVERDOSAGE

If puffy eyelids are observed, this is a symptom or over-hydration and the therapy may need to be discontinued temporarily.

170mm(w) x 197mm (h)

Black 100%

Up-to-date information on the treatment of over-dosage can be obtained from The National Poison Centre, Universiti Sains Malaysia.

TOXICOLOGY

Pregnancy/Reproduction
Problems in humans have not been documented.

Breast feeding
Problems in humans have not been documented. Continued breast feeding during the treatment and maintenance phases of oral rehydration therapy is vital for the management of diarrhoea.

PAEDIATRICS

Although oral rehydration therapy appears to be safe and effective in neonates, it has not been evaluated in premature infants. The range of sodium concentrations recommended is 40 to 60mEq per litre for maintenance solutions and 75 to 90mEq per litre for rehydration solutions. To allow adequate intake of free water in the prevention of hypernatreamia with the use of Uphalyte, feeding (including breast milk) may continue and/or the infant may be given a separate feeding of plain water after every 2 doses of Uphalyte solution.

GERIATRICS

Carbohydrate and electrolyte solutions are well tolerated by elderly patients.

SIDE EFFECTS

Mild vomiting may occur when oral therapy has begun, but therapy should be continued with frequent, small amount of solution administered slowly. Rarely, symptoms of hypernatreamia (dizziness, fast heartbeat, high blood pressure, irritability, muscle twitching, restlessness, seizures, swelling of feet or lower leg, or weakness) may be experienced.

CONTRAINDICATIONS

Precise parenteral administration of water and electrolytes is recommended in the following conditions and the use of oral rehydration should not be used except under special circumstances: Anuria or oliguria, severe dehydration with symptoms of shock, severe diarrhoea, inability to drink, severe and sustained vomiting.

Diarrhoea is exacerbated and dehydration worsened when oral rehydration solutions are given to patients with glucose malabsorption; volume of stool is greatly increased and contains large amounts of glucose. Rehydration therapy should be discontinued. In patients with intestinal obstruction, paralytic ileus or perforated bowel, delayed passage carbohydrate and electrolytes solution through the gastrointestinal tract may increase the risk the gastrointestinal irritation.

DRUG INTERACTIONS

None reported.

PRECAUTIONS

To be used with care in patients with impaired renal function, high blood pressure, diabetes or heart ailments. Patient monitoring may be important. Blood pressure measurements are recommended to detect shock due to severe dehydration. Body weight is to be checked periodically to determine the degree of rehydration or the recurrence of dehydration. Serum electrolytes and pH measurements are recommended to help determine status of individual ions and acid-base status. Glucose malabsorption tests are important when oral rehydration solutions appears to exacerbate diarrhoea; infant's faeces should be monitored for reducing substances to detect transient monosaccharide malabsorption which may occur during acute infectious diarrhoea; sugar intake should be eliminated or decreased if glucose intolerance is present; intravenous fluid therapy may be required. Patients with severe diarrhoea should be observed for signs of rehydration every 3-6 hours, i.e., normal skin turgor, normal urine flow, normal pulse rate and volume, and a sense of well-being. Stool volume measurements are to be done periodically to determine the dose and continued need for maintenance therapy; the volume of ingested replacement solution should equal the volume of stool losses. If stool volume cannot be determined, an intake of 10-15mL of rehydration solution per kg of body weight per hour is suggested.

PRESENTATION

Pack of 50's sachets.
(Nett weight : 7.84gm per sachet – orange
7.76gm per sachet – natural)

STORAGE CONDITIONS

Store in a dry place below 30°C.
Protect from light.
Keep out of reach of children.
Jauhi daripada kanak-kanak.
Shelf life: please refer to outer package.

Route of administration: Oral

Product Registration Holder:
Duopharma Manufacturing (Bangi) Sdn. Bhd.
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